SETTING UP MEDBURY AESTHETICS

Lagos first. Five businesses, one division. The case in two pages.

The ask. NGN 25M for a three month build and a two month handover across five companies, in three payments: NGN 15M to start, NGN 5M when the division is trading and NGN 5M at the switch to management, the last two released only against measured outcomes. Government and regulatory fees of about NGN 3.1M are absorbed inside that number, so what CFA earns is under NGN 22M.

What is being set up

Five separate operating companies sharing one market, one supply chain and one patient. Four of the five are built from nothing. Only Medlyfe Regenerative and Longevity trades today and it is nascent, so it is the one warm list the division starts with. Lyfe Plastics and Dermatology is another of the five and already has its own memorandum and shareholders.

Entity	What it is
Medlyfe Regenerative and Longevity	Regenerative medicine, longevity programmes and infusion. Already trading but nascent, and the only one of the five with patients today. Programme based and pre-sellable, so revenue can be taken before the doors open. Highest revenue per square metre on the campus
Bariatric and Metabolic	Medical weight management. Long programmes, high retention, feeds pharmacy and laboratory directly
Cosmetic Dentistry	Veneers, whitening, alignment and implants. High ticket, cash paying, and the most natural cross-sell in the division: the patient having her face treated wants her smile to match
Hair Restoration	Single patient days, instruments brought by the operating partner, very little fixed cost
Lyfe Plastics and Dermatology	The surgical end, with medical and surgical dermatology alongside it. Its own tri-party shareholding, its own clinical director, its own memorandum, already drafted

Why it is a division and not five clinics

One rate card stops five businesses pricing against each other in the same building. One clinical record means the woman who arrives for one of the five is known to the other four without being acquired a second time. One procurement framework buys devices once and routes the clinical supply to Medbury's own pharmacy and diagnostics. Set them up separately and you get five small clinics with five cost bases.

None of that is true on paper, so a platform is being built to make it true. One clinical record across the five, room and diary scheduling so they share space without colliding, programme and course management so treatment can be sold and collected before it is delivered, deposits and instalments reconciled to the gateway, imaging and consent held against the record, batch and expiry on every injectable, the enquiry funnel, and a board dashboard produced by the system rather than by hand. It is configured, migrated, staffed and live before opening, and it transfers to Abuja already built.

What the work actually is

Not registration. Company formation is a week of somebody's time. The work is the operating build: the pricing architecture, the clinical governance and protocol set, the technology platform, the service partners under signed service levels, the funnel, the front office and revenue cycle, and the governance rhythm the Board will read. Per entity, the scope and menu, the staffing model, the equipment specification, the clinical brief the space is built to, the launch sequence, and commissioning before the first real patient rather than after.

The pricing work is the largest single item and the one most often done badly. Primary price research taken at the point of enquiry rather than off websites, willingness to pay tested with the actual segment, cost to serve built bottom up per procedure, then sensitivity tested against volume at sixty per cent of planned fill, a realistic naira band, mix, elasticity and ramp. A rate card without that model behind it is a hope, and it cannot be corrected later: you do not raise prices on a base you acquired at the wrong number.

Built once, used twice

Almost none of this is a Lagos asset. The rate card, the procurement framework, the protocols, the configured platform, the partner terms and the standard operating procedures are written once and transfer to Abuja when it is ready. Only the facility, the team and the licences are local. Build it cheaply here and Abuja has to be built a second second time.

What it is worth

Where the value is	NGN M	How it arises
The pricing architecture	20	5 points of margin on modelled division revenue of NGN 400M. Set once, compounds every year
Revenue before opening	25	Regenerative, longevity and weight management sell as programmes, so they can be contracted and paid for before the doors open
The Abuja build not repeated	18	Abuja re-papers the operating system rather than rebuilding it
Cross-sell between the five	recurring	A patient acquired once is reachable by four more businesses at no further acquisition cost, but only if the division is designed for it at the start
Against a fee of	**25**	**The first three exceed it inside year one and two of them repeat**

Supply, pharmacy and diagnostics are deliberately absent. Medbury owns those businesses and any margin the division produces there belongs to the group. CFA will route the demand internally and write the service levels, but will not present internal capture as value it brokered.

How the fee is released

When	NGN M	What has to be true
On signing	15	Mobilisation. The team stands up and is paid from week one. The only payment not tied to an outcome
1		A rate card that survives its sensitivities, delivered with the research and the model behind it. Internal supply routed and specified
2		NGN 25M of programme revenue contracted and collected before opening. Primary and named second source signed in every critical category
3	5	First entity lawfully trading and first revenue banked, every partner under a signed SLA with a numeric measure, cross-sell working. Released only if every measure above it has also been met
6	5	At the switch to management. Handover complete, the division on the normal management rhythm, and the measures still holding
Total	**25**	

Three payments. NGN 15M to start, because the team is paid before any measure can be met. NGN 5M when the division is trading and only if every measure has been met. NGN 5M at the switch to management in month 6, and only if it is still trading to the same measures once the sprint is over.

Where the 25 comes from

	NGN M	Note
Abuja, as quoted	45	Included CFA project management on the facility build
Less that project management fee	(20)	Your contractors manage the Lagos build
Comparable scope	**25**	**The operating build, across five entities rather than one**

A day rate is the wrong test. What matters is that the identified value is NGN 63M against a fee of NGN 25M; that a rate card set five points light costs NGN 20M a year and cannot be corrected; and that hiring a division lead, a pricing analyst, a regulatory hand and a recruiter for five months costs more than this before anyone has negotiated anything, leaves you carrying the team afterwards, and leaves nothing written down to transfer to Abuja.

What Medbury provides, and what the fee excludes

	Detail
Medbury provides	A single decision maker who can approve a rate card, a supplier and an SLA without a second meeting. The facility on a programme CFA can plan the launch against. Funding against the approved budget. Approval of the operating lead CFA recruits. Group access to referrals, laboratory, pharmacy and the relationships that open a partner theatre
The fee excludes	Project management of the facility build, which is why this is 25 and not 45, and which is quoted separately if Medbury would rather CFA managed it. Equipment and stock, which are capital. Salaries of the team recruited, who are the company's employees from day one. Hardware, connectivity and the payment gateway charges behind the platform, where the build, configuration, migration and training are in the fee but the terminals and the network are the company's. The shareholder negotiation for Lyfe Plastics and Dermatology, separately mandated. The management of the division from month 6, which is agreed and sits on its own contract rather than inside this fee

Who owns what is built

Everything produced under this mandate belongs to the company: the rate card and the model underneath it, the protocols, the standard operating procedures, the partner agreements, the platform configuration, the patient data and the integrations. CFA's own platforms are licensed to the division for the term at a rate agreed before we start, and if that licence ever ends the company keeps its data and configuration in a form it can carry elsewhere.

Setup and management are one arrangement

CFA manages Medbury Aesthetics from month 6, as agreed: two chief operating officers employed by Medbury and seconded to CFA, a support fee of NGN 3.0m a month, and 3 per cent of collected net revenue where the Board's goals are met. The handover months are a transition, not an exit, and it is why the build is priced where it is: I am building something I then have to run.

The four decisions

- **The fee.** NGN 25M against the tranches above.
- **The start date.** Three months from mobilisation. A start in the next fortnight puts first revenue inside the year.
- **The management agreement.** Terms agreed and issued separately, with the KPI schedule and budget process drafted during the sprint.
- **The operating lead.** CFA recruits, the Group approves, in post by month 4.

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